

Behçet Disease — The Silk Road Bazaar (Ulcers, Uveitis & Vascular Danger)



1. Silk Road banner

WHAT YOU SEE

SILK ROAD map banner, Turkey, 20–40 yrs

WHAT IT ENCODES

Behçet is a variable-vessel vasculitis that affects arteries AND veins of any size, and it clusters along the ancient Silk Road — highest prevalence in Turkey (where it is most common), the Middle East, and East Asia/Japan. Typical onset is age 20–40, and disease tends to be MORE severe in young men. Turkish prevalence approaches ~1 in 250.

BOARD TRAP

Silk Road / Turkish ancestry in a young adult is the classic demographic hook — it is NOT a northern-European disease, so a 'Behçet' answer in a 70-year-old Scandinavian woman should make you reconsider.

2. Oral ulcers ×3/yr

WHAT YOU SEE

Mouth ulcer on figure, 'ORAL 3 of 4', most sensitive

WHAT IT ENCODES

Recurrent painful oral aphthous ulcers (≥3 episodes in 12 months) are the most COMMON and effectively required feature, present in nearly all patients. In the International Criteria for Behçet's Disease (ICBD), oral aphthosis scores points alongside genital ulcers, ocular, skin, vascular, and neuro involvement (≥4 points = diagnosis). Mnemonic: mouth first, most sensitive.

BOARD TRAP

Oral ulcers are nearly universal — their ABSENCE makes Behçet very unlikely. Don't be fooled into picking Behçet for a patient who has genital ulcers and uveitis but has NEVER had oral aphthae.

3. Genital ulcers (scarring)

WHAT YOU SEE

Genital ulcer label, '~50%', 'most specific'

WHAT IT ENCODES

Recurrent genital ulcers (scrotum in men, vulva in women) occur in roughly half of patients and characteristically heal with SCARRING — a useful exam clue. They are the most SPECIFIC of the mucocutaneous features, whereas oral ulcers are the most sensitive. Distinguish from herpes (viral culture/PCR negative in Behçet).

BOARD TRAP

Genital ulcers are the most SPECIFIC criterion and they SCAR — confusing 'most specific' (genital) with 'most sensitive' (oral) is the classic swap, and attributing the scarring ulcers to recurrent HSV is the infectious-mimic trap.

4. Uveitis (panuveitis)

WHAT YOU SEE

An inflamed red eye on the figure labeled PANUVEITIS 50% / hypopyon 10% — the layer of pus pooled at the bottom of the iris IS the sterile hypopyon, marking sight-threatening panuveitis.

WHAT IT ENCODES

Ocular disease occurs in ~50% and is sight-threatening: classically bilateral relapsing PANUVEITIS with retinal vasculitis, and a sterile HYPOPYON (layered pus in the anterior chamber) in ~10–20%. Posterior/retinal involvement carries the highest risk of permanent vision loss. Treat aggressively with systemic immunosuppression (steroids + azathioprine, infliximab, or other anti-TNF).

BOARD TRAP

A STERILE hypopyon with posterior uveitis demands aggressive systemic therapy, not topical drops alone — treating it like routine anterior uveitis (topical steroids only) leads to blindness.

5. Pathergy

WHAT YOU SEE

A red needle-prick papule on the skin tagged acne/pathergy — the skin erupting at the prick site IS pathergy, the exaggerated neutrophilic hyperreactivity.

WHAT IT ENCODES

Pathergy is an exaggerated, nonspecific skin hyperreactivity: a sterile needle prick produces an erythematous papule or pustule read at 24–48 hours. It reflects the underlying neutrophilic hyperinflammation (which also drives acneiform lesions and erythema nodosum-like nodules). A positive test supports diagnosis and scores in the ICBD criteria.

BOARD TRAP

A positive pathergy test SUPPORTS Behçet but is neither required nor sensitive — it is far less common (and less positive) in Western/Silk-Road-distant patients, so a NEGATIVE pathergy test does NOT exclude the disease.

6. HLA-B51

WHAT YOU SEE

HLA B51 placard, 'not diagnostic'

WHAT IT ENCODES

HLA-B51 (B5 split) is the strongest known genetic association and correlates with more severe disease (ocular, vascular), but it is found in many healthy people from endemic regions. It is an epidemiologic association, NOT a confirmatory test — Behçet remains a clinical diagnosis based on criteria. Mnemonic: B51 = bad (severity), but not the test.

BOARD TRAP

HLA-B51 is an ASSOCIATION, not a diagnostic test — ordering HLA-B51 'to confirm Behçet' (the way one might check HLA-B27 reasoning) is wrong; a positive result neither rules it in nor a negative rules it out.

7. Venous thrombosis

WHAT YOU SEE

A red-rope 'VASCULAR DANGER ZONE' barrier with clotted veins labeled Budd-Chiari and cerebral venous — the inflamed, clogged vessels ARE the venous thromboses at unusual sites.

WHAT IT ENCODES

Behçet is one of the few vasculitides with prominent VENOUS disease: inflammation of the vessel WALL causes thrombosis at unusual sites — DVT, superficial thrombophlebitis, Budd-Chiari (hepatic vein), cerebral venous sinus thrombosis, and vena cava thrombosis. The clot is densely adherent to an inflamed wall, so it rarely embolizes (PE is uncommon).

BOARD TRAP

The thrombosis is driven by INFLAMMATION of the vessel wall, NOT a primary hypercoagulable state — a hypercoagulability workup (Factor V Leiden, etc.) is the wrong reflex, and because clots are adherent, embolization to PE is actually uncommon.

8. Immunosuppress, not anticoagulate

WHAT YOU SEE

'INFLAMMATORY not hypercoagulable — IMMUNOSUPPRESS' banner

WHAT IT ENCODES

Because Behçet clots arise from vessel-wall inflammation, the cornerstone of treating thrombosis is IMMUNOSUPPRESSION (high-dose corticosteroids plus azathioprine, cyclophosphamide, or anti-TNF) to quiet the wall. Anticoagulation is at most adjunctive and controversial, and is dangerous if coexisting arterial aneurysms are present.

BOARD TRAP

Treat Behçet venous thrombosis with IMMUNOSUPPRESSION first — choosing 'start heparin/warfarin' as the primary therapy is wrong, and anticoagulating a patient who actually has pulmonary artery aneurysms can cause fatal hemorrhage.

9. Pulmonary artery aneurysms

WHAT YOU SEE

Lungs with PA ANEURYSMS, 'high mortality'

WHAT IT ENCODES

Pulmonary ARTERY aneurysms are a hallmark arterial complication and a leading cause of Behçet death; rupture into the airway produces massive HEMOPTYSIS. They are managed with intensive immunosuppression (steroids + cyclophosphamide) and sometimes embolization — never anticoagulation.

BOARD TRAP

Hemoptysis in a Behçet patient = pulmonary ARTERY aneurysm, NOT pulmonary embolism — the dangerous trap is treating the imaging filling defect as a PE and anticoagulating, which can precipitate fatal aneurysm bleeding.

10. GI involvement

WHAT YOU SEE

GI panel, ileocecal deep ulcers, perforation risk

WHAT IT ENCODES

Intestinal Behçet produces deep, punched-out ulcers favoring the ILEOCECAL region that can bleed or PERFORATE, closely mimicking Crohn disease both clinically and endoscopically. Behçet ulcers tend to be fewer, rounder, and deeper, and lack the transmural granulomas/skip-pattern typical of Crohn.

BOARD TRAP

Ileocecal ulcers that look like Crohn are the GI mimic — the discriminator is the EXTRA-intestinal Behçet picture (recurrent oral + genital ulcers, uveitis, Silk Road ancestry); calling it 'Crohn' and missing the systemic vasculitis is the error.

11. Neuro-Behçet

WHAT YOU SEE

A brain under a 'NEURO TENT' split into vascular vs parenchymal halves — the two-compartment brain IS the dual pattern of neuro-Behçet (sinus thrombosis vs brainstem lesions).

WHAT IT ENCODES

Neuro-Behçet has two patterns: PARENCHYMAL (inflammatory brainstem/diencephalic lesions, the more common and more disabling form, MRI shows mesodiencephalic involvement) and NON-parenchymal/VASCULAR (cerebral venous sinus thrombosis presenting with headache/papilledema). Both mark severe disease requiring systemic immunosuppression.

BOARD TRAP

Brainstem parenchymal disease and cerebral venous sinus thrombosis define SEVERE Behçet needing systemic immunosuppression — managing a CVST here as a routine clot (anticoagulation alone) without immunosuppression undertreats the underlying vasculitis.

12. Treatment pharmacy

WHAT YOU SEE

Treatment shelf: colchicine, topical, azathioprine, anti-TNF

WHAT IT ENCODES

Therapy is severity-tiered. Mucocutaneous (oral/genital ulcers, skin): topical steroids, colchicine, and apremilast. Major-organ disease (eye, CNS, vascular, GI): systemic corticosteroids PLUS a steroid-sparing immunosuppressant — azathioprine, cyclophosphamide, cyclosporine, or anti-TNF (infliximab/adalimumab); interferon-alpha is an option for refractory ocular disease.

BOARD TRAP

Colchicine/apremilast control mucocutaneous flares but are INADEQUATE for sight- or life-threatening organ involvement — answering 'colchicine' for a patient with panuveitis, neuro, or vascular disease undertreats and risks blindness/death.